

300 mm

180 mm

166 mm

14 mm

For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

Abilimac 15 (Aripiprazole Tablets 15 mg)

COMPOSITION:
Each uncoated tablet contains
Aripiprazole 15 mg

Product Description
Abilimac 15: Yellow, round shaped, uncoated tablets debossed with 'CL 75' on one side and plain on the other side.

Pharmacodynamics
ATC Code: N05AX12
Aripiprazole exhibits high affinity for dopamine D2 and D3, serotonin 5-HT1A and 5-HT2A receptors (Ki values of 0.34, 0.8, 1.7, and 3.4 nM, respectively), moderate affinity for dopamine D4, serotonin 5-HT2C and 5-HT7, alpha1-adrenergic and histamine H1 receptors (Ki values of 44, 15, 39, 57, and 61 nM, respectively), and moderate affinity for the serotonin reuptake site (Ki=98 nM). Aripiprazole has no appreciable affinity for cholinergic muscarinic receptors (IC50>1000 nM). Aripiprazole functions as a partial agonist at the dopamine D2 and the serotonin 5-HT1A receptors, and as an antagonist at serotonin 5-HT2A receptor.
The mechanism of action of aripiprazole, as with other drugs having efficacy in schizophrenia, bipolar disorder, and agitation associated with schizophrenia or bipolar disorder, is unknown. However, it has been proposed that the efficacy of aripiprazole is mediated through a combination of partial agonist activity at D2 and 5-HT1A receptors and antagonist activity at 5-HT2A receptors. Actions at receptors other than D2, 5-HT1A, and 5-HT2A may explain some of the other clinical effects of aripiprazole, e.g., the orthostatic hypotension observed with aripiprazole may be explained by its antagonist activity at adrenergic alpha1 receptors.

Pharmacokinetics
Absorption:
Aripiprazole is well absorbed, with peak plasma concentrations occurring within 3-5 hours after dosing. Aripiprazole undergoes minimal pre-systemic metabolism. The absolute oral bioavailability of the tablet formulation is 87%. There is no effect of a high fat meal on the pharmacokinetics of aripiprazole.
Distribution:
Aripiprazole is widely distributed throughout the body with an apparent volume of distribution of 4.9 l/kg, indicating extensive extravascular distribution. At therapeutic concentrations, aripiprazole and dehydro-aripiprazole are greater than 99% bound to serum proteins, binding primarily to albumin.
Metabolism:
Aripiprazole is extensively metabolised by the liver primarily by three biotransformation pathways: dehydrogenation, hydroxylation, and N-dealkylation. Based on in vitro studies, CYP3A4 and CYP2D6 enzymes are responsible for dehydrogenation and hydroxylation of aripiprazole, and N-dealkylation is catalysed by CYP3A4. Aripiprazole is the predominant medicinal product moiety in systemic circulation. At steady state, dehydro-aripiprazole, the active metabolite, represents about 40% of aripiprazole AUC in plasma.

Elimination:
The mean elimination half-lives for aripiprazole are approximately 75 hours in extensive metabolisers of CYP2D6 and approximately 146 hours in poor metabolisers of CYP2D6. The total body clearance of aripiprazole is 0.7 ml/min/kg, which is primarily hepatic. Following a single oral dose of [14C]-labelled aripiprazole, approximately 27% of the administered radioactivity was recovered in the urine and approximately 60% in the faeces. Less than 1% of unchanged aripiprazole was excreted in the urine and approximately 18% was recovered unchanged in the faeces.

Indication
Abilimac is indicated for the treatment of acute episodes of schizophrenia and for maintenance of clinical improvement during continuation therapy. Abilimac is indicated for the treatment of acute manic episodes associated with Bipolar I Disorder.

Recommended Dose
Schizophrenia
The recommended starting dose for Aripiprazole is 10 or 15 mg/day administered on a once-a-day schedule without regard to meals. The maintenance dose for Aripiprazole is 15 mg/day. Doses in the range of 10 to 30 mg/day have been established as effective.
Bipolar mania
The recommended starting and target dose for Aripiprazole is 15 mg given once a day, without regards to meals. The dose can be increased to 30 mg/day based on clinical response. Dose adjustments, if indicated, should occur at intervals of not less than 24 hours. Antimanic efficacy (3-12 weeks) was demonstrated in doses of 15 or 30 mg/day. The safety of doses above 30 mg/day has not been evaluated.
Special populations
Renal impairment
No dosage adjustment is required in patients with renal impairment.
Hepatic impairment
No dosage adjustment is required for patients with hepatic impairment (Child-Pugh Class A, B, or C)
Pediatric
The safety and efficacy of Aripiprazole in patients under 18 years of age have not been established.
Elderly
No dosage adjustment is required for patients ≥ 65 years of age. However, experience with this patient population is limited.
Gender
No dosage adjustment is required for female patients relative to male patients.
Patients taking medications metabolized by CYP2D6 or 3A4
Dosage adjustment for patients taking aripiprazole concomitantly with potent CYP3A4 or CYP2D6 inhibitors: When concomitant administration of a potent CYP3A4 or CYP2D6 inhibitor with aripiprazole occurs, the aripiprazole dose should be reduced to one-half of the usual dose. When the CYP3A4 or CYP2D6 inhibitor is withdrawn from the combination therapy, the aripiprazole dose should then be increased.
Dosage adjustment for patients taking potent CYP3A4 inducers: When a potent CYP3A4 inducer is added to aripiprazole therapy, the aripiprazole dose should be doubled. Additional dose increases of aripiprazole should be based on clinical evaluation. When the CYP3A4 inducer is withdrawn from the combination therapy, the aripiprazole dose should be reduced.
Consideration should be given to reducing the daily dose in individual patients who are on multiple concomitant medications that inhibit CYP3A4 and CYP2D6 enzymes.
Smoking Status
No dosage adjustment is required for smoking patients relative to non-smoking patients.

Route of Administration
Oral

Contraindications
Hypersensitivity to the active substance or to any of the excipients.

Warnings and Precautions
During antipsychotic treatment, improvement in the patient's clinical condition may take several days to some weeks. Patients should be closely monitored throughout this period.
The occurrence of suicidal behaviour is inherent in psychotic illnesses and mood disorders and in some cases has been reported early after initiation or switch of antipsychotic therapy, including treatment with aripiprazole.
Close supervision of high-risk patients should accompany antipsychotic therapy.
There was no increased risk of suicidality with aripiprazole compared to other antipsychotics among patients with schizophrenia or bipolar disorder.
Cardiovascular disorders: Aripiprazole should be used with caution in patients with known cardiovascular disease (history of myocardial infarction or ischaemic heart disease, heart failure, or conduction abnormalities), cerebrovascular disease, conditions which would predispose patients

to hypotension (dehydration, hypovolemia, and treatment with antihypertensive medicinal products) or hypertension, including accelerated or malignant.
Venous thromboembolism (VTE) have been observed with antipsychotic drug use. Since patients treated with antipsychotics often present with acquired risk factors for VTE, all possible risk factors for VTE should be identified before and during treatment with aripiprazole and preventive measures undertaken.
Conduction abnormalities: Aripiprazole should be used with caution in patients with a family history of QT prolongation.
Tardive dyskinesia: If signs and symptoms of tardive dyskinesia appear in a patient on aripiprazole, dose reduction or discontinuation should be considered. These symptoms can temporally deteriorate or can even arise after discontinuation of treatment.
Neuroleptic Malignant Syndrome (NMS): NMS is a potentially fatal symptom complex associated with antipsychotic medicinal products. Rare cases of NMS were reported during treatment with aripiprazole. Manifestations of NMS are hyperpyrexia, muscle rigidity, altered mental status and evidence of autonomic instability (irregular pulse or blood pressure, tachycardia, diaphoresis and cardiac dysrhythmia). Additional signs may include elevated creatine phosphokinase, myoglobinuria (rhabdomyolysis), and acute renal failure. However, elevated creatine phosphokinase and rhabdomyolysis, not necessarily in association with NMS, have also been reported. If a patient develops signs and symptoms indicative of NMS, or presents with unexplained high fever without additional clinical manifestations of NMS, all antipsychotic medicinal products, including aripiprazole, must be discontinued.
Seizure: Uncommon cases of seizure were reported during treatment with aripiprazole. Therefore, aripiprazole should be used with caution in patients who have a history of seizure disorder or have conditions associated with seizures.
Elderly patients with dementia-related psychosis:
Increased mortality: In elderly patients with psychosis associated with Alzheimer's disease, patients treated with aripiprazole were at increased risk of death. Although the causes of deaths were varied, most of the deaths appeared to be either cardiovascular (e.g. heart failure, sudden death) or infectious (e.g. pneumonia) in nature.
Cerebrovascular adverse reactions: Cerebrovascular adverse reactions (e.g. stroke, transient ischaemic attack), including fatalities, was observed with Aripiprazole use. There is a significant dose response relationship for cerebrovascular adverse reactions while on Aripiprazole. Aripiprazole is not indicated for the treatment of dementia-related psychosis.
Hyperglycaemia and diabetes mellitus: Hyperglycaemia, in some cases extreme and associated with ketoacidosis or hyperosmolar coma or death, has been reported in patients treated with atypical antipsychotic agents. Assessment of the relationship between atypical antipsychotics use and glucose abnormalities is complicated by the possibility of and increased background risk of diabetes mellitus in patients with schizophrenia and the increasing incidence of diabetes mellitus in general population. Given this confounders, the relationship between atypical antipsychotic use and hyperglycamia related adverse events is not completely understood. However, epidemiological studies suggest an increase risk of treatment-emergent hyperglycemia-related adverse events in patients treated with the atypical antipsychotics. Precise risk estimates for hyperglycemia-related adverse events in patients treated with atypical antipsychotics are not available.
Patients with an established diagnosis of diabetes mellitus who are started on atypical antipsychotics should be monitored regularly for worsening of glucose control. Patients with risk factors for diabetes mellitus (e.g. obesity, family history of diabetes) who are starting treatment with atypical antipsychotics should undergo fasting blood glucose testing at the beginning of treatment and periodically during treatment. Any patient treated with atypical antipsychotics should be monitored for symptoms of hyperglycemia including polydipsia, polyuria, polyphagia, and weakness. Patients who develop symptoms of hyperglycemia during treatment with atypical antipsychotics should undergo fasting blood glucose testing. In some cases, hyperglycemia has resolved when the atypical antipsychotic was discontinued; however, some patients required continuation of anti-diabetic treatment despite discontinuation of the suspect drug.
Hypersensitivity: as with other medicinal products, hypersensitivity reactions, characterised by allergic symptoms, may occur with aripiprazole.
Weight gain: weight gain is commonly seen in schizophrenic and bipolar mania patients due to comorbidities, use of antipsychotics known to cause weight gain, poorly managed life-style, and might lead to severe complications.
Dysphagia: oesophageal dysmotility and aspiration have been associated with antipsychotic treatment, including aripiprazole. Aripiprazole and other antipsychotic active substances should be used cautiously in patients at risk for aspiration pneumonia.
Pathological gambling and impulse-control problems: patients can experience increased urges, particularly for gambling, and the inability to control these urges while taking aripiprazole. Other urges, reported include: increased sexual urges, compulsive shopping, binge or compulsive eating, and other impulsive and compulsive behaviours.
It is important for prescribers to ask patients or their caregivers specifically about the development of new or increased gambling urges, or other urges, while being treated with aripiprazole. It should be noted that impulse-control symptoms can be associated with the underlying disorder; however in some cases urges were reported to have stopped when the dose was reduced or the medication was discontinued. Patients who are at higher risk for impulse-control problems (e.g. personal or family history of obsessive-compulsive disorder, impulse-control disorder, bipolar disorder, impulsive personality, alcoholism, drug abuse or other addictive behaviours) would require closer monitoring for new or worsening of uncontrollable urges. Impulse-control problems may result in harm to the patient and others if not recognized. Consider dose reduction or stopping the medication if a patient develops such urges while taking aripiprazole.
This product contains lactose from bovine milk.
Interactions with Other Medicaments
Due to its α1-adrenergic receptor antagonism, aripiprazole has the potential to enhance the effect of certain antihypertensive agents.
Given the primary CNS effects of aripiprazole, caution should be used when aripiprazole is taken in combination with alcohol or other CNS medicinal products with overlapping adverse reactions such as sedation.
If aripiprazole is administered concomitantly with medicinal products known to cause QT prolongation or electrolyte imbalance, caution should be used.
Potential for other medicinal products to affect aripiprazole:
A gastric acid blocker, the H2 antagonist famotidine, reduces aripiprazole rate of absorption but this effect is deemed not clinically relevant.
Aripiprazole is metabolised by multiple pathways involving the CYP2D6 and CYP3A4 enzymes but not CYP1A enzymes. Thus, no dosage adjustment is required for smokers.
Aripiprazole dose should be reduced to approximately one-half of its prescribed dose when concomitant administration of aripiprazole with quinidine occurs. Other potent inhibitors of CYP2D6, such as fluoxetine and paroxetine, may be expected to have similar effects and similar dose reductions should therefore be applied.
In CYP2D6 poor metabolisers, concomitant use of potent inhibitors of CYP3A4 may result in higher plasma concentrations of aripiprazole compared to that in CYP2D6 extensive metabolizers. When considering concomitant administration of ketoconazole or other potent CYP3A4 inhibitors with aripiprazole, potential benefits should outweigh the potential risks to the patient. When concomitant administration of ketoconazole with aripiprazole occurs, aripiprazole dose should be reduced to approximately one-half of its prescribed dose. Other potent inhibitors of CYP3A4, such as itraconazole and HIV protease inhibitors, may be expected to have similar effects and similar dose reductions should therefore be applied.
Upon discontinuation of the CYP2D6 or 3A4 inhibitor, the dosage of aripiprazole should be increased to the level prior to the initiation of the concomitant therapy.
When weak inhibitors of CYP3A4 (e.g., diltiazem or escitalopram) or CYP2D6 are used concomitantly with aripiprazole, modest increases in aripiprazole concentrations might be expected.
Aripiprazole dose should be doubled when concomitant administration of aripiprazole occurs with carbamazepine. Other potent inducers of CYP3A4 (such as rifampicin, rifabutin, phenytoin, phenobarbital, primidone, efavirenz, nevirapine and St. John's Wort) may be expected to have similar effects and similar dose increases should therefore be applied. Upon discontinuation of potent CYP3A4 inducers, the dosage of aripiprazole should be reduced to the recommended dose.
When either valproate or lithium were administered concomitantly with aripiprazole, there was no significant change in aripiprazole concentrations.
Potential for Aripiprazole to affect other medicinal products:
Aripiprazole and dehydro-aripiprazole did not show potential for altering CYP1A2-mediated metabolism in vitro. Thus, aripiprazole is unlikely to cause clinically important medicinal product interactions mediated by these enzymes.
When aripiprazole was administered concomitantly with either valproate, lithium or lamotrigine, there was no clinically important change in valproate, lithium or lamotrigine concentrations.

